

Table 15.11.2.1.1.2: Treatment-Emergent Adverse Events: Overall Summary
Safety Analysis Set
STARS Generated Output

Number (%) of Participants with	Drug A (N=50)	Drug B (N=50)	Total (N=100)
Any TEAE	50 (100.0%)	50 (100.0%)	100 (100.0%)
TEAE with Grade 3 or Higher	43 (86.0%)	42 (84.0%)	85 (85.0%)
TEAE with Grade 2 or Higher	50 (100.0%)	49 (98.0%)	99 (99.0%)
TEAE Related to Study Drug	49 (98.0%)	42 (84.0%)	91 (91.0%)
TEAE Related to Study Drug with Grade 3 or Higher	33 (66.0%)	27 (54.0%)	60 (60.0%)
TEAE Related to Study Drug with Grade 2 or Higher	49 (98.0%)	41 (82.0%)	90 (90.0%)
TE Serious AE	39 (78.0%)	34 (68.0%)	73 (73.0%)
TE Serious AE Related to Study Drug	27 (54.0%)	17 (34.0%)	44 (44.0%)
TEAE Leading to Premature Discontinuation of Study Drug	26 (52.0%)	25 (50.0%)	51 (51.0%)
TEAE Leading to Dose Interruption of Study Drug	31 (62.0%)	25 (50.0%)	56 (56.0%)

TE = treatment-emergent; TEAE = treatment-emergent adverse event.

Adverse events were coded according to MedDRA Version {XX.X}.

Treatment-emergent events began on or after the study drug start date up to {XX} days {or study drug-specific therapeutic reach} after the date of the last dose of study drug [and prior to the day of initiation of subsequent anticancer/antiviral/etc therapy].

Severity grades were defined by [Gilead Grading Scale for Severity of Adverse Events and Laboratory Abnormalities][The CTCAE Version X][The DAIDS Version XX]. [Participants were counted only one based on their highest grade for each summary row.]

Death includes any death that occurred during the study. Percentages for TEAE Dose Limiting Toxicity was calculated based on the number of

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Page 1 of 2

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Number (%) of Participants with	Drug A (N=50)	Drug B (N=50)	Total (N=100)
TEAE Leading to Dose Reduction of Study Drug	25 (50.0%)	24 (48.0%)	49 (49.0%)
TEAE Leading to Premature Discontinuation of Study	4 (8.0%)	3 (6.0%)	7 (7.0%)
TEAE Leading to Death	26 (52.0%)	25 (50.0%)	51 (51.0%)

TE = treatment-emergent; TEAE = treatment-emergent adverse event.

Adverse events were coded according to MedDRA Version {XX.X}.

Treatment-emergent events began on or after the study drug start date up to {XX} days {or study drug-specific therapeutic reach} after the date of the last dose of study drug [and prior to the day of initiation of subsequent anticancer/antiviral/etc therapy].

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